

Policy on Hospital Formulary

«Hospital Name»

Document control

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Prepared by	Drug and Therapeutics Committee Chair Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The hospital formulary at «Hospital Name» is developed collaboratively, kept current, made available to every clinician, and followed — with clear procedures for acquiring both formulary and non-formulary medications.

A formulary is an approved list of medications for use in the hospital. Drugs on the formulary are generally those determined to be both medically effective and cost-effective for the patients the hospital treats.

1. Purpose

This policy explains how «Hospital Name» develops and updates its formulary, makes it available to clinicians, monitors adherence to it, and follows defined procedures for acquiring formulary and non-formulary medications.

This policy does not cover medication storage, prescription, or dispensing in detail — those are covered in other hospital policies.

2. Scope

This policy applies to the multi-disciplinary medication management committee, pharmacy staff, and all treating doctors at «Hospital Name».

3. Policy standards

«Hospital Name»'s multi-disciplinary committee develops the formulary to match the organisation's mission, patient needs, and scope of services, reviews and updates it at least once a year, and makes the current version available to every treating doctor. Clinicians' adherence to the formulary is monitored, and defined procedures govern how formulary and non-formulary medications are acquired.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not use a formulary that was not collaboratively prepared by the multi-disciplinary committee, or that omits the molecule name, formulation, and strength for each entry, or that leaves out implants and devices.
2. Do not let the formulary go more than a year without a collaborative review and update.
3. Do not leave the current formulary unavailable to any treating doctor.
4. Do not acquire formulary medications without a documented procedure covering vendor selection, vendor evaluation, reorder levels, the indenting process, purchase order generation, receipt of goods, and managing stock-outs.
5. Do not use a non-formulary medication through a local purchase without following an evaluation, authorisation, and ratification process, including a decision on whether it should be added to the formulary.

5. What we do

5.1 Develop the formulary collaboratively

The multi-disciplinary committee prepares the formulary to include medications necessary for the organisation's mission, patient needs, and scope of services. The National List of Essential Medicines or the WHO Model List of Essential Medicines could inform this, along with factors like harm potential, drug interactions, and likelihood of patient safety incidents. The committee could also consider a system-wise or speciality-wise formulary.

At minimum, every formulary entry names the molecule, its formulation, and its strength. «Hospital Name» works to limit the number of concentrations of any one drug in the formulary. Implants and devices are treated as drugs and included in the formulary.

5.2 Review and update the formulary

The committee reviews and updates the formulary collaboratively at least once a year — this could cover all medications or focus on certain categories, and could be done speciality by speciality. Non-formulary drugs regularly procured the previous year could be added during this review. Patient safety factors — adverse drug reactions, changing disease or resistance patterns, and cost — could inform the review.

5.3 Make the current formulary available

The current formulary is available to every treating doctor at «Hospital Name», in physical or electronic form.

5.4 Monitor adherence to the formulary

«Hospital Name» makes sure prescriptions follow the formulary, and monitors how often prescriptions are rejected or a local purchase is needed because a non-formulary drug was prescribed.

5.5 Follow the procedure for acquiring formulary medications

A documented procedure governs vendor selection, vendor evaluation, reorder levels, the indenting process, generating purchase orders, and receiving goods, and also addresses how stock-outs are managed.

5.6 Follow the procedure for non-formulary medications

Where a medication is not in the formulary — for example through a local purchase or a hotline arrangement for urgent need — «Hospital Name» follows a process of evaluation, authorisation, and ratification, and makes a decision on whether that medication should be added to the formulary.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the formulary is in place and that acquisition and non-formulary procedures are followed.
Drug and Therapeutics Committee Chair	Owns formulary development and annual review; chairs the committee that governs all formulary decisions.
Multi-disciplinary committee (DTC)	Develops, reviews and updates the formulary collaboratively; approves non-formulary requests; monitors adherence.
Pharmacy In-Charge	Implements the acquisition procedure; manages non-formulary procurement records; ensures the current formulary is available at all prescribing locations.
Treating doctors / clinicians	Prescribe from the current formulary; follow the non-formulary request process where a non-formulary drug is required.
Quality Coordinator	Monitors formulary adherence; audits this policy; holds training records and staff acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
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Formulary development	Collaboratively prepared by the committee; every entry names molecule, formulation, and strength; implants and devices included as drugs.
Annual review	Conducted and documented by the committee at least once a year; additions, deletions, and restrictions minuted with clinical reason.
Formulary availability	Current version available to every treating doctor in physical or electronic form; outdated versions removed.
Adherence monitoring	Frequency of non-formulary prescriptions, rejections, and local purchases tracked and reported to the committee.
Acquisition procedure	All six procedure elements present and in use: vendor selection, vendor evaluation, reorder levels, indenting, purchase order generation, and receipt of goods; stock-out management addressed.
Non-formulary process	Evaluation, authorisation, and ratification documented for every local purchase of a non-formulary medication; formulary-addition decision recorded.

8. Training and staff acknowledgement

The multi-disciplinary committee, pharmacy staff, and all treating doctors shall be familiar with the formulary, the formulary acquisition procedure, and the non-formulary medication process.

I have read the Policy on Hospital Formulary of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to the multi-disciplinary committee, pharmacy staff, all treating doctors, the Medication Safety Officer, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
DTC	Drug and Therapeutics Committee (the

	organisation's multi-disciplinary medication management committee)
MSO	Medication Safety Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
NLEM	National List of Essential Medicines
MOM	Management of Medication (NABH Hospitals chapter)
WHO	World Health Organization

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.2.a	CORE	Sections 3 and 5.1 address collaborative formulary preparation by the committee, minimum content (molecule, formulation, strength for every entry), and the inclusion of implants and devices as drugs.
MOM.2.b	Commitment	Sections 3 and 5.2 address the collaborative annual review and update of the formulary, including the review of all or certain medication categories.
MOM.2.c	Commitment	Sections 3 and 5.3 address making the current formulary available to every treating doctor in physical or electronic form.
MOM.2.d	Achievement	Section 5.4 addresses monitoring of prescription adherence to the formulary and tracking of rejections and local purchases.
MOM.2.e	Commitment*	Sections 3 and 5.5 address the documented acquisition procedure covering all six elements: vendor selection, vendor evaluation, reorder levels, indenting process, purchase order generation, receipt of goods, and stock-

		out management.
MOM.2.f	Commitment*	Sections 3 and 5.6 address the non-formulary medication process — evaluation, authorisation, and ratification — and the decision on formulary inclusion.

12. Required Records/Evidence Checklist

Formulary development — MOM.2.a (CORE)

1. Formulary prepared collaboratively by the multi-disciplinary committee.
2. Every formulary entry naming the molecule, formulation, and strength.
3. Formulary including implants and devices as drugs.
4. Record showing the committee limited the number of concentrations of any one drug where possible.

Annual review — MOM.2.b

1. Record of the committee's collaborative annual review and update of the formulary.
2. Additions, deletions, and restrictions minuted with clinical reasons.
3. Dated current-formulary cover showing the version in use.

Formulary availability — MOM.2.c

1. Current formulary available at prescribing locations — OPD consulting rooms, wards, ICU, emergency, OT, and the hospital information system or intranet.
2. Record showing outdated copies removed when a new version is issued.

Adherence monitoring — MOM.2.d

1. Prescription-versus-formulary audit records.
2. Tracking records for prescriptions rejected or resulting in local purchase because a non-formulary drug was prescribed.
3. DTC records of any department's repeated non-adherence tabled for review.

Acquisition procedure — MOM.2.e

1. Documented acquisition procedure covering vendor selection, vendor evaluation, reorder levels, indenting process, purchase order generation, and receipt of goods.
2. Record showing how stock-outs are managed under the procedure.
3. Procurement records showing approved supplier, receipt quality checks, and batch and expiry logged.

Non-formulary process — MOM.2.f

1. Written non-formulary request record for every local purchase — including clinical justification (evaluation), approval (authorisation), and committee ratification.

2. Record of the decision on whether the non-formulary item should be added to the formulary.
3. Same-shift retrospective documentation record for any emergency non-formulary use.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.2.
2. Ministry of Health and Family Welfare, Government of India. (2022). <https://main.mohfw.gov.in/newshighlights-104> National List of Essential Medicines (NLEM) 2022.
3. World Health Organization. (2023). <https://www.who.int/publications/i/item/WHO-MHP-HPS-EML-2023.02> WHO Model List of Essential Medicines, 23rd list.

Disclaimer

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